

Comprehensive Operational Report: Aizer Health Pediatric Department

1. Departmental Mission, Scope, and Site Architecture

Site-Specific Operational Profiles

- Forest Road Location (Strategic Hub):
 - **Address:** 49 Forest Rd.
 - **Routing:** Standard "Peds Lab" and "Ped Nurse" designations.
- Baker Town Location:
 - **Address:** 48 Baker Town Rd.
 - **Operational Distinction:** All nursing and laboratory services utilize the **"BK" prefix** (e.g., BK Peds Lab). This is a strategic administrative tool used to ensure distinct routing and prevent cross-site diagnostic errors in the patient hub.
- Blooming Grove Location:
 - **Address:** 1200 Route 208.
 - **Availability:** Monday–Thursday only; exclusively staffed by Dr. Dina Faiden.
 - **Operational Note:** While the system utilizes a "208-Lab" routing designation for administrative processing, this site **physically lacks on-site laboratory facilities**. All specimens must be routed externally.

Strategic Scope Analysis

The department's tiered structure dictates a strict triage logic. Blooming Grove serves as a localized satellite for routine visits, whereas the Forest Road hub is reserved for complex cases and multidisciplinary care. This hierarchical footprint allows us to maintain a nimble community presence while concentrating expensive clinical assets at a high-volume center, effectively managing departmental overhead.

2. Clinical Protocols and Diagnostic Pathways

Standardization of clinical pathways is essential for mitigating clinical risk and ensuring regulatory and financial compliance. Adherence to these rigid protocols protects the health of our most vulnerable patients while safeguarding the department's revenue cycle.

Well-Visit and Physical Exam (PE) Governance

- **Commercial/Private Insurance:** Adherence to the "one calendar year plus one day" rule is mandatory to prevent insurance claim denials.
- **Managed Care (Fidelis) Exceptions:** Early physicals are permitted only if:
 - It has been at least six months since the last PE.
 - The child has reached the **next age milestone** (e.g., turned 6 years old).
- **Newborn (NB) Protocols (6 weeks or less):**
 - **Scheduling:** Mandatory booking at the **start of the provider's shift** to optimize clinical hygiene and minimize exposure to sick-visit traffic.
 - **Documentation:** Staff must request hospital discharge papers. For Good Samaritan (GS) births, the hearing screening lab ID must be recorded in the reason for visit.
 - **Billing/Administrative Alerts:** All NB visits must include "NPP" (New Patient Paperwork) or "MRC" alerts.

Diagnostic and Vaccination Protocols

- **Immunization Workflows:**
 - all patient calls requesting immunization should be directed to Marisa Kraft or Jeanette Alcantara.
- **Tetanus Clinical Mandate:** Regardless of PE status, all Tetanus administrations require a provider check-up immediately prior to the shot to assess acute injury risk.
- **Lab and Imaging Workflow:** A strict "Order First" policy is in effect. Staff must verify orders in the Patient Hub, then direct the call to Marisa Kraft or Jeanette Alcantara. **Under no circumstances** may administrative staff provide medical advice or test results via phone; these must be routed to the "Q-Pediatrics Nursing Inquiries" queue.

3. Patient Care, Specialist Services, and Provider Matrix

The department's provider mix is strategically aligned with the community's linguistic and cultural needs, utilizing proficiencies in Spanish, Yiddish, and Hebrew to enhance patient engagement.

Comprehensive Provider Matrix

Provider, Title, Booking Nuances, Clinical Niche / Demographic Restriction

- **Dr. Adam Polinger, MD**, Double/Triple booking; 20/10 min block rule., GS Affiliated; Spanish/Hebrew/Yiddish. Comfortable with teenage females.
- **Dr. Eliezer Frommer, MD**, "Strict, fast-paced; 10 min (≤ 1yr); 20 min (>1yr).", GS Affiliated; Yiddish. Not comfortable with teenage females.
- **Dr. Lazar Khaimov, DO**, Double/Triple booking for sick visits and PE., High volume. Not comfortable with teenage females.
- **Dr. Chana Heintz, MD**, No double booking for Physicals., Only provider for stitches. Not comfortable with teenage males.
- **Dr. Tamar Dachoh, MD**, 20-min physicals; sibling double booking okay., Works most week (no Fridays). Not comfortable with teenage males.
- **Dr. Dina Faiden, MD**, Formerly Donna Dick. Follows template strictly., Spanish; 20-min PE (all ages). Not comfortable with teenage males.
- **Robin Aschkenasy, PA**, 20-min physicals; sibling double booking okay., Yiddish; No Fridays. Not comfortable with teenage males.
- **Lily Namanworth, NP**, NO double/triple booking (except siblings), Spanish/Yiddish. Not comfortable with teenage males.
- **Lori Lambert-Derario, NP**, Starts shift double-booked; max 30 pts/day., Non-Jewish; Tue/Fri nights. Not comfortable with teenage males.

Sub-Specialty Integration (Forest Road Only)

- **Note:** Dr. Hochfelder/Siegel prioritize the first slot of the shift/post-lunch for **Food Challenges**.
- **Specialist Constraints:**
 - **Dr. Cooper (Cardiology):** No United Healthcare. MVP requires secondary Medicaid.
 - **Dr. Welter (Pulmonology):** Rotates the **2nd last Tuesday** of every month.
 - **Dr. Gottlieb (Rheumatology):** No United Healthcare.

4. Administrative Infrastructure and Staffing Framework

The efficiency of our administrative "backbone" relies on clear communication hierarchies and a specialized triage system for clinical inquiries.

Nursing and Administrative Rosters

All immunization and lab requests must be directed to Marisa or Jeannette for chart auditing.

- **Marisa Kraft:** Sun 9-5; Mon-Thu 12-8.
- **Jeannette Alcantara:** Mon-Thu 9-5 (Auditing/Scheduling until 4 PM); Fri 9-2.
- **Administrative Protocol:** If Marisa or Jeannette are available, perform a **"soft transfer."** If unavailable, send a Telephone Encounter (TE).
- **Additional Coverage:** Alexa (Mon-Thu 10-6; alternate Sundays) and Lizzette (Tue/Fri 9-5).

Communication and Referral Management

- **Sally Carilli (Ext. 1934):** Routes all **Controlled Substance Refills** (Adderall, Concerta, Guanfacine, Strattera, Intuniv, Ritalin, Vyvanse, Focalin, Xanax, Alprazolam) and mental health follow-ups.
- **Anisa Azeez (Ext. 1911):** Directs referrals and 2020 Transportation forms.
- **Haley Newton (Ext. 1909):** Coordinates pediatric specialty scheduling.

Registration and Confirmation Logic

- **Phone Number Search:** Always search by phone number first to identify linked family accounts.
- **Arrival Instruction (Mandatory):** Advise all patients to arrive *before* their appointment time and document this in "General Notes."
- **Real-Time Operational Oversight:** The confirmation color-coding system acts as a real-time audit trail:
 - **White:** No confirmation.
 - **Blue/Green:** Automated/Manual message left.
 - **Purple (with 'V'):** Staff-confirmed. This transition from automated attempts to manual "V" confirmation is a key metric in reducing "no-show" rates.

5. Financial Compliance and Insurance Management

Insurance Verification Matrix

Eligibility status indicators provide immediate visibility into the financial viability of a visit:

- **Green (Y):** Eligible.
- **Yellow (Y):** Active, but Aizer is not the primary care provider (PCP).
- **Black (?):** Pending verification.
- **Red (X/I):** Inactive or error in data.

Plan-Specific Regulations

- **United Healthcare (UHC):** Commercial plans require online prior authorization for specialists.
- **Healthfirst:** Accepted **only** if the patient has active Medicaid as a secondary payer.
- **Relationship Entry:** For all Medicaid/Managed Care, "Relationship to Insured" must be set to **Self (1)**.
- **Self-Pay:** Sliding Fee Scale starts at \$25 (income-based/1-year validity); Flat Self-Pay is \$100.

6. Operational Challenges, Metrics, and Future Considerations

Resource and Inventory Controls

- **Medication Dispensing:** OTC medications (Tylenol/Motrin) are insurance-covered **only if dispensed before patient checkout**. Post-checkout requests require the patient to purchase OTC.
- **HIPAA and Forms:** School forms are faxed only if the PE is UTD. External school inquiries require the institution to be added to the patient's HIPAA authorization.

Operational Optimization

The department utilizes "grouped scheduling" for 5-minute physicals. By placing two 5-minute physicals on the same line, we maximize "acute care availability" by opening subsequent lines for up to three sick visits. This grouping strategy is our primary lever for handling seasonal surges in patient volume.